

disease	protocol	goal	MNT																																
Nephrotic syndrome	E=30-35Kcal/kg Protein: 0.8-1g/kg Fat: less than 30%	—	Micronutrients: ensure adequate intake, monitor for signs vitamin D def. • Na: no-added-salt diet • K : monitor as serum levels may increase with ACE inhibitors • Fluid restriction may be necessary, depending on the response to diuretics																																
CKD	K: Restriction until K is >5.5 mg/day • <2.4g/day • 3-4g for CKD • 2-3 for ESRD Ph: Restriction until ph. is >4.6 mg/day • 800-1000mg • RDA 700-1250MG • Average intake:	1. delay the progression of renal failure 2. To manage the symptoms associated with the syndrome (edema, hypoalbuminemia, and hyperlipidemia) 3. To delay the need for dialysis 4. Maintain & improve nutritional status 5. Treat hypertension 6. Restore & maintain electrolyte imbalance 7. Restore & maintain micronutrients imbalance (minerals & vitamins. e.g. Fe, Vit D, ph, ca)	Protocol: <table><tr><th></th><th>E</th><th>P</th><th>F</th><th>Na</th><th>K</th><th>Ph</th><th>Ca</th></tr><tr><td>Without dialysis</td><td>30-35 Kcal</td><td>Stage 1-2= 0.8-1 g= 12-15% Stage 3-4= 0.6-0.75 g= 10%</td><td>As disired</td><td>Var.</td><td>Var.</td><td>0.8-1.2g</td><td>800-1000</td></tr><tr><td>Hemo-dialysis</td><td>35</td><td>1.2g/kg</td><td>750-1000 +urine output</td><td>1.5-2g</td><td>2-3g</td><td>0.8-1.2g</td><td>800-1000</td></tr><tr><td>Perito-nial</td><td>30-35 kcal</td><td>1.2-1.5g/kg</td><td>750-1000 +urine output</td><td>1.5-4g</td><td>3-4g</td><td>0.8-1.2g</td><td>800-1000</td></tr></table>		E	P	F	Na	K	Ph	Ca	Without dialysis	30-35 Kcal	Stage 1-2= 0.8-1 g= 12-15% Stage 3-4= 0.6-0.75 g= 10%	As disired	Var.	Var.	0.8-1.2g	800-1000	Hemo-dialysis	35	1.2g/kg	750-1000 +urine output	1.5-2g	2-3g	0.8-1.2g	800-1000	Perito-nial	30-35 kcal	1.2-1.5g/kg	750-1000 +urine output	1.5-4g	3-4g	0.8-1.2g	800-1000
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	1400-2000 mg	8. Maintain growth in children	
Nephron-lithiasis	Fluid: 2-3 l/day Protein: 1g/day Ca: not less than 700-800mg/day Na: 2300mg/day		• that soft drinks and colas that contain phosphoric acid should also be avoided - Excessive animal protein intake modulate several urinary risk factors. • Low Oxalates Diet: The recommendation for tea drinkers is to drink only a moderate amount of weak tea (about 2 c per day) with milk. • Purines: Low purine diet is recommended. • Citrate is a urinary stone inhibitor - Lemon juice should be encouraged in patients with low urinary citrate • Other: intake of potassium, magnesium, fiber, fruit and vegetables is associated with decreased stone risk
ARF	E: 30-40Kcal Protein: 0.8-1g		• K: 3050mEq • Na: 20-40mEq

	Fluid: output+ 500ml		
cancer	E: •25- 30 For cancer inactive, nonstress, sepsis. •30-35 For cancer nut. Repletion, wt. gain ,hema-topoietic. •30kcal For cancer hypermetabolic, stress. Protein: 1.5-2g/day Fluid: 20-40ml/kg	• Prevent or minimize weight changes as patients are in hypermetabolic state and greatest losses are from protein stores and body fat • Diminish toxicity of treatment correct cachexia • Prevent depletion of immunity • Reduce side effects of cancer therapy • Control complications such as anemia or multiple organ failure	• Counter side effects of therapy (radiation, surgery) • When inadequate intake and/or increased losses of micronutrients are suspected, a multivitamin and mineral supplement may be appropriate. e.g. antioxidants. e.g. vit A, C, E, Zinc, Selenium • Special attention should be given to food containing folic acid e.g. spinach, avocado, orange juice, dried beans & fortified cereals • Regular nutritional assessment is required
Brain cancer	-	-	• To control brain edema: limit sodium to 3g/day • If there is dysphagia perform diet texture modification

			***Ketogenic diet
Colorectal carcinoma	-	• Correct anemia (due to bleeding). • If ileal resection: Vit B 12 deficiency may occur and bile salts may be lost in stool.	• Limit foods that cause gases such as corn, cauliflower, broccoli, cabbage, • Decrease intestinal residue by following a low fiber diet. • Decrease animal sources of protein as colorectal carcinoma is related to red meat